LabSense evidence summary

10 November 2026. Printed from the evidence page of https://lsa-health.github.io/ : what was tested, on what, and what is still in development. Synthetic cases only, plus one de-identified real case for value extraction. This is not clinical validation. A supervised clinical evaluation is being planned; its dates depend on site and ethics approvals.

EVIDENCE AND STATUS, 10 NOVEMBER 2026

Evaluation evidence and current capabilities

LabSense is a Clinical Investigation System (CIS) for clinical decision support (CDS), in active development. This site shows a recorded run of the features tested so far; further features are being validated. It works under physician oversight and does not diagnose. The full chain is tested on synthetic cases written for testing, and value extraction has also been compared on one de-identified real case, where the full chain is being prepared; none of it is clinical validation. A supervised clinical evaluation is being planned; its dates depend on site and ethics approvals.

- ● RECORDED — output of a test run on a synthetic case
- ● EVALUATED — in a measured test
- ● LIVE IN THE WORKSPACE — on the local system, not online
- ● BEING VALIDATED — built in part, checks under way
- ◐ IN DEVELOPMENT — not shown, work under way
- ○ PLANNED — scheduled; work has not started
- ◐ BUILT, SWITCHED OFF — in the code, off by default, not run live

Feature status · SYN30 run record · Earlier tests · Why LabSense · AI, security and clinical status · Files and checksums

Feature status

CAPABILITY	STATUS	BASIS
Reading printed rows with their unit, printed interval and collection date	• RECORDED	SYN30MP1; an earlier reading test on 46 synthetic PDFs
Identity check on the first report; later reports for another patient ID refused before storage	• RECORDED	SYN30MP1 (confirmation); two earlier synthetic cases (refusal)
Stored record with every printed row stored or named with a reason	• RECORDED	SYN30MP1: 109 printed rows, 107 stored, 2 named
Reading every panel of the current report (multi-panel reading), the evaluator default	• RECORDED	SYN30MP1 and its repeat on the integrated build, 9 November 2026
A critical value on the current report driving the finding and urgency	• RECORDED	SYN30MP1: “Critical Potassium (6.4 mmol/L)”, Emergency, in both stages, after reload and in both exports
History per test, against the printed interval, units kept apart	• RECORDED	SYN30MP1
Targeted questions, scripted answers, receipts of what changed and what did not	• RECORDED	SYN30MP1
Reopening the original report from a new browser	• RECORDED	SYN30MP1: all nine opened as whole PDFs; byte-for-byte match shown in an earlier run

CAPABILITY	STATUS	BASIS
Source verification of each stored value against its printed row, with checksums	• BEING VALIDATED	Not shown on this site; every stored value is tied to its printed row and report
Clicking a value to highlight its row on the printed page	◐ IN DEVELOPMENT	Not shown; a report opens as the whole document
Review snapshot export of the first reading and the reassessment	• RECORDED	SYN30MP1: 30 files each, primary concern equal to the screen
AI model reading first, deterministic reader as a dissent-only check	◐ BUILT, SWITCHED OFF	Built and switched off; no live model run on SYN30
AI wording of questions	◐ BUILT, SWITCHED OFF	Wording hook off; reassessment and final analysis are deterministic and have no AI component
Supervised sessions on synthetic cases, on one local machine	• LIVE IN THE WORKSPACE	Behind a sign-in; not online
Remote use	○ PLANNED	Not permitted until a lifetime for backend credentials is built

SYN30 run record

Recorded run SYN30MP1 and its repeat, 9 November 2026

SYN30 is one synthetic patient at a fictional laboratory. The case was written before the files were generated. The run went from sign-in and upload to export and a rebuild in a new browser, on one local machine, with the deterministic reader only and no AI model call. A repeat run, SYN30MP2, gave identical results.

9 reports	**30** printed pages	**109** printed result rows
107 stored	**2** named with a reason	**7** dates, March 2024 to September 2026

2024-03-05
1 report, 4 pages
First report; whose report it is confirmed here

2024-09-17
1 report, 3 pages
Routine follow-up

2025-03-11
2 reports, 8 pages
Original, and a reprint (4 pages) counted once

2025-08-26
2 reports, 6 pages
Original, and an amended copy (3 pages) with a changed creatinine range

2026-01-20
1 report, 3 pages
Creatinine in µmol/L; some results printed as limits

2026-05-14
1 report, 2 pages
Report date only, no collection date

2026-09-08
1 report, 4 pages
Latest: potassium 6.4 mmol/L, creatinine printed twice

Where the 109 printed rows went

ROWS	WHAT HAPPENED TO THEM
107	Stored. Four of them are limits kept exactly as printed (for example TSH <0.01), not turned into numbers.
2	Not stored, each named with its reason: a second creatinine on the latest report (1.62 mg/dL; neither value chosen) and a white cell count hidden by a covered cell.
8	Stored but kept out of every trend: total cholesterol on six report copies, creatinine 80 µmol/L (in this run, kept as printed in its own unit series; the recorded build gave a transcription check as the reason, but the cause was a unit difference, corrected in a later build) and creatinine 1.04 mg/dL (held for review).

What the run did, step by step

STEP	WHAT HAPPENED
Upload and reading	All nine reports read and confirmed, uploaded out of date order.

STEP	WHAT HAPPENED
Identity	A person confirmed the first report; the other eight matched it.
Stored record	Rebuilt in a new browser: 107 stored rows and 2 named rows, for 17 tests.
First reading	Led by “Critical Potassium (6.4 mmol/L)”, urgency Emergency, evidence support Moderate.
Questions and answers	Three questions; scripted answers No, No, Yes from a signed-in synthetic account.
Reassessment	The Yes changed familial dyslipidaemia to “inherited/atherogenic risk raised”; the finding and urgency did not change, and the screen said so.
Reload and originals	Results unchanged after a reload; all nine originals reopened as whole PDFs in a new browser.
Exports	A snapshot of each reading, 30 files each, with the same primary concern as the screen.
Access	Reads that skipped the sign-in gateway were refused; reads by another role were refused or returned nothing.

Technical details

Counting: the record screen's heading reads "103 stored values" because it counts numbers only; the 4 printed limits (eGFR >90, TSH <0.01, urine albumin/creatinine ratio <3, ferritin <5, all on the report of 2026-01-20) are stored as text and listed under it, and the run's retrieval check counts 107 stored rows. 107 stored plus 2 named equals the 109 result rows printed on the nine PDFs, with the reprint and the amended report counted as printed. The first reading read 13 of the 19 value-bearing lines of the latest report, as LabSense counts lines; that report prints 15 result rows.

Configuration: every panel of each report read (multi-panel reading), the configuration used for evaluator sessions.

Timings on the server: reading 6 to 15 s per report; record read at most 264 ms; consult median 375 ms, slowest 766 ms; reopening 47 to 125 ms per document; 2.8 s per export. Driver step times include fixed waits and other work ran on the same machine, so these are upper bounds.

Builds: backend integ/sb22-security c0c5e123 (later commits to 954e7a32 change AI cost handling only), frontend aa06e68 (configuration 884766d makes multi-panel reading the default in the controlled evaluator build), gateway bd866d28.

Repeatability (SYN30MP2 against SYN30MP1): screens, stored record, questions and answers identical, and exports identical once times and IDs are removed.

Earlier tests

In an earlier reading test on 46 synthetic PDFs across 15 cases (late October 2026), the conventional reader read every printed value line into a stored row, 243 lines in all; the one file with no rows was a report printed for another patient, refused by design.

• EVALUATED

Reports printed for another patient were refused before storage in browser test runs of two earlier synthetic cases.

• RECORDED

Beyond the synthetic cases, value extraction has also been compared on one de-identified real multi-year case: the AI reader against the conventional reader, 251 observations, checked against a frozen reference set (August 2026). This covers extraction only; the full question-and-export chain on real reports is being prepared.

• EVALUATED

In an earlier recorded run (8 November 2026), a new browser reopened all nine originals and each matched the printed PDF byte for byte (SHA-256 compared). • RECORDED

Why LabSense

Reading a report is common. Keeping a whole history honest is the work.

The mechanisms below are shown today on synthetic cases, each with how it works. The items listed after them are not shown yet; each carries its status.

One history per test

Reports from different dates and panels become one dated series per test.

Mechanism: printed name, unit, the laboratory's own interval, collection date and file kept per value; series join only the same test in the same unit.

• RECORDED

Holds back instead of guessing

A value it cannot trust is kept out, with the reason on screen.

Mechanism: printed limits kept as limits; two values on one report, neither chosen; a blank cell named.

• RECORDED

Checks whose report it is

A person confirms identity once; reports printed for another patient ID are refused before storage.

Mechanism: the confirmed printed ID becomes the record's anchor (shown in earlier synthetic cases).

• RECORDED

Says what it did not assess

The assessment lists the results it did not read, so a gap is visible instead of silent.

Mechanism: coverage computed against the stored record for the report's date.

• RECORDED

A receipt for every answer

After an answer, the screen states what changed and what did not.

Mechanism: answers applied to named elements only; unchanged fields are stated, not implied.

• RECORDED

The original stays the original

Every report reopens exactly as printed, and each stage of an encounter exports as a snapshot that names the build it came from.

Mechanism: retained source files compared by SHA-256; a replay manifest names the build.

• RECORDED

In development and planned

Source verification of each stored value against its printed row	• BEING VALIDATED
Clicking a value to highlight its row on the printed page	◐ IN DEVELOPMENT
A multi-centre library of laboratory report formats, each checked row by row	◐ IN DEVELOPMENT
A disagreement calibration database: reader disagreements and how clinicians resolved them	◐ IN DEVELOPMENT
Laboratory quality reports on missing intervals and duplicates	◐ IN DEVELOPMENT
Unit conversion: under the conversion rule ratified by the Lead Clinician on 10 November 2026, creatinine 80 µmol/L corresponds to 0.90 mg/dL, with the printed value kept; not yet applied by the application. ☐ ILLUSTRATION Illustration of the clinically approved conversion rule. The recorded run kept this value as printed in its own unit series; the application will apply the rule after activation is verified.	◐ IN DEVELOPMENT
Hospital record integration (FHIR) and other evidence sources	○ PLANNED

Built, switched off, not yet run live

An AI model reading first, with the deterministic reader as an independent check that can only raise a disagreement. The path exists in the code and is off by default; a first live run on synthetic cases is planned within its cost limits.

Reading one report at a time, and LabSense across a patient's reports

Mechanisms only. This is not a comparison with any product.

WHAT IS CHECKED	READING ONE REPORT AT A TIME	LABSENSE ACROSS A PATIENT'S REPORTS
Printed rows	The reviewer reads each report on its own and keeps track of which rows were read.	Every printed row is counted, and either stored or named with the reason it was not stored (SYN30MP1: 109 rows, 107 stored, 2 named).

WHAT IS CHECKED	READING ONE REPORT AT A TIME	LABSENSE ACROSS A PATIENT'S REPORTS
Doubtful values	When a report prints two values for one test, or an unexpected unit, the reviewer decides which to use.	A value it cannot trust is held back with the reason, instead of one being chosen.
Whose report it is	The reviewer checks the name and ID printed on each report.	The patient identity is checked before anything is stored; a later report printed for another ID is refused.
Answers	What a patient's answer changed lives in the reviewer's own notes.	A receipt states what each answer changed and what it did not.

The designed reading path has an AI model read first, with the deterministic reader as an independent check that can only raise a disagreement; the recorded SYN30 run used the deterministic reader alone.

Where others are ahead: laboratory networks, distribution, regulatory status, clinicians in the loop and integrations (see Comparison with other products).

About this recorded run

- **What the film shows.** At 0:55 the narrator says creatinine 80 µmol/L was kept as printed and excluded from the mg/dL series. The film and its captions are unchanged.
- **The reason given.** The recorded build labelled this value as refused by its transcription check. The real cause was a unit difference (µmol/L against mg/dL); a later build keeps such values as printed in their own unit series.
- **Where the label shows.** The same label appears in the History capture in the walkthrough, and the same kind of reason was given for the six total cholesterol rows (a mmol/L unit difference). Each of those places points here.
- **Case provenance.** The worked case is a purpose-built synthetic case: a fictional patient and laboratory, built on 7 November 2026 from written expectations, with nine reports from 2024-03-05 to 2026-09-08. The de-identified real case was used for value extraction only (251 observations).
- **Who answered.** Every answer in the recorded run was scripted and clicked by a signed-in synthetic evaluator account, not by a clinician or a patient. In a later run the exports recorded the answers as not attributed, so this site makes no claim about answer attribution.

- **Status today.** The Lead Clinician ratified the conversion table on 10 November 2026. The application does not apply it yet, so the comparable figure on these pages is an illustration of the rule, not a recorded result.

AI, security and clinical status

AI

The SYN30 runs used the deterministic reader only, with no model call. The intended design for reading is an AI model first, with the deterministic reader as an independent check that can only raise a disagreement; that path is built, switched off by default and not yet run with a live model on SYN30. Question choice, answer handling and the final analysis are deterministic in this build; the optional AI wording of questions is switched off.

Security and access

LabSense runs on one machine, bound to the local network interface only, behind a sign-in gateway. Requests that bypass the gateway are refused, cross-case and cross-role reads are refused, and signed-out uploads are refused; these were checked in the SYN30 run and in the gateway test suites. Remote use, even with synthetic reports, is not permitted until a lifetime for backend credentials is built. Real reports are not accepted.

Clinical

A licensed clinician who heads the LabSense clinical governance panel co-signed a limited set of rules on 29 September 2026, for example that where a laboratory panel lacks a ratified definition, individual rows may be shown but no overall panel normality may be asserted. That is approval of specific rules. No clinician has reviewed the outputs shown on this website, nor the general test descriptions on the results cards.

Comparison with other products

A reading of the public material of seven products on 29 October 2026 (consumer testing services, report explainers, a clinical literature assistant and a record-exchange network). It is not a tested comparison; their figures are their own claims and were not checked. We did not find the behaviours shown here described in that material; that is an absence of evidence, not proof that the products lack them.

Clinical validation: A supervised clinical evaluation is being planned; its dates depend on site and ethics approvals.

Real-patient use: none; one de-identified real case used for an extraction comparison

Regulatory clearance: none

Pilot: none agreed

Users or customers: none yet

Head-to-head comparison: not run

Files and checksums

The nine synthetic reports of SYN30, byte-identical to the files the run uploaded. Anyone can compare a reopened report with these checksums.

REPORT	SHA-256
2024-03-05	40694b26a7a51df95d39ade1abdf701ef10bb7472f7739ab2d065cbfe7eacf41
2024-09-17	da7b120a1f944f551f323016ff296f89ec4880086ca0895aa3ccf064c5e72ee0
2025-03-11	bdaea3465b2a0baab9d412a6c1a5f43bf45c4060cbe6cc36516a434af3ad4fe2
2025-03-11 reprint	0c54d62e4e66b04e68ec1d486a3cd1df22321f89a5737d9e8f80d58dc3f6ea59
2025-08-26	db99e7e53313a9d9f29d92093f5c812408d5c37fc684f05d74018cc3b731dd2c
2025-08-26 amended	da5e333b25e0bc456840e68f614b23ed64bd2aab11446652a2bc200c8569ac01
2026-01-20	3d4ccf3a228fc24b21a46e6ce11e048aac058f7f6680f4df1fd52aa38bcb68ff
2026-05-14	507e0207d77c7dbb946ba2fbefd34fb991015153be6e3e144480b4707b35c2f4
2026-09-08	d93d0b409303bee94a572e4926059ce51843ec02d09e85ce841621d4af16cec6

Full screen captures from SYN30MP1 (1440 px)

Review screen · First reading · Reassessment · History · Results not assessed · Record in a new browser · Originals reopened

The exports themselves are not published: they contain local machine paths from the test computer.

LabSense supports a clinician’s assessment and does not make a diagnosis. Not clinically validated; no regulatory clearance. Every case shown is synthetic: fictional patients and a fictional

laboratory. Not for routine clinical care until clinical evaluation is complete. This website stores nothing, sets no cookies and loads nothing from other sites.

Evidence and status Request a supervised evaluation